

Discharge Criteria Guide

[When to discharge](#) · [When to continue](#) · [How to document](#) · [Medicare Part B](#) · 2026

Discharge decisions require clinical judgment balanced with Medicare coverage rules. Discharging too early creates patient risk. Continuing past the point of skilled need creates compliance risk. This guide helps you make and document that decision correctly.

Discharge — YES vs NO

✓ Appropriate to Discharge When

- Goals have been met and patient is at or near prior level of function
- Patient has reached maximum functional potential — no further gains expected
- Skilled need no longer exists — patient/caregiver can safely continue independently
- Patient refuses therapy consistently — document thoroughly and refer to IDT
- Patient is medically unstable and cannot participate in therapy safely
- Patient is being discharged from the facility to home or higher level of care
- Maintenance program established and restorative nursing can carry it forward
- Patient is non-compliant and goals cannot be achieved with current level of participation

✗ Not Appropriate to Discharge When

- Patient has not met goals but still has functional potential remaining
- Skilled monitoring is required to prevent deterioration (Jimmo standard)
- Patient has a progressive condition requiring ongoing clinical adjustment
- Patient recently had a significant change in status or setback
- Caregiver not yet trained to safely manage patient's program
- Medical complication arises that creates new skilled need
- Goals were met but new functional deficits have been identified
- Discharge would create an unsafe transition — safety net still required

The 4 Discharge Scenarios

Goals Met — Full Discharge

When to use: Patient has achieved all stated goals and returned to or near prior level of function. Skilled need no longer exists. Patient and/or caregiver independent with HEP or maintenance activities.

Document: Document: goals met, current functional status, prior level comparison, HEP provided, caregiver training completed, reason skilled care no longer required.

Maximum Plateau — Discharge to Restorative

When to use: Patient has reached a functional plateau with no expectation of further improvement. Skilled monitoring is not required. Restorative nursing can maintain the current program safely.

Document: Document: current functional level, plateau rationale, why restorative is appropriate, restorative program handed off, goals achieved vs not achieved and why.

Skilled Need Continues — Transition to Maintenance

When to use: Patient at plateau but skilled monitoring IS still required to maintain function or prevent decline. Do NOT discharge — transition to a maintenance program with continued skilled oversight.

Document: Document per Jimmo standard: what function is maintained, what decline risk exists, why non-skilled staff cannot perform or monitor the program.

Patient Discharge from Facility

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When to use: Patient is leaving the facility. Therapy must complete a formal discharge summary and ensure continuity of care at the next level.

Document: Document: final functional status, goals met/not met, discharge destination, outpatient therapy referral if needed, HEP provided, equipment needs, caregiver training.

Discharge Note — What Must Be Included

✓ Reason for discharge	Goals met / plateau / patient request / medical instability / facility discharge
✓ Initial vs final functional status	Compare admission status to discharge status for each goal area
✓ Goals met / partially met / not met	Document each goal individually with status and rationale
✓ Skilled need rationale (if continuing)	If transitioning to maintenance — full Jimmo documentation
✓ Home exercise program	Document HEP provided, patient/caregiver education, and their ability to perform
✓ Equipment needs	Any DME ordered or recommended — walker, wheelchair, splint, adaptive equipment
✓ Caregiver training	What was taught, who was trained, and demonstrated competency
✓ Referrals	Outpatient therapy, home health, restorative nursing handoff
✓ Prognosis	Patient's expected long-term functional trajectory

Am I Ready to Discharge? — Pre-Discharge Self-Assessment

Patient Name:	_____	Date:	_____
Discipline:	PT / OT / ST	Discharge Type:	Goals Met / Plateau / Maintenance / Facility D/C

Clinical Readiness

☐	Have all therapy goals been addressed — met, partially met, or formally closed with rationale?	Yes / No / N/A
☐	Has the patient reached prior level of function OR has maximum therapeutic benefit been achieved?	Yes / No / N/A
☐	Is there no remaining skilled need that requires continued therapy intervention?	Yes / No / N/A
☐	If on a maintenance program — has skilled need been reassessed and confirmed as no longer required?	Yes / No / N/A
☐	Has the patient's functional status been re-assessed and documented within the past 3 days?	Yes / No / N/A

Patient & Caregiver Preparation

☐	Has the patient/caregiver been educated on the HEP and demonstrated understanding?	Yes / No / N/A
☐	Has caregiver training been completed for any assist techniques required at home?	Yes / No / N/A
☐	Does the patient/caregiver know signs of decline and when to seek further therapy?	Yes / No / N/A

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☐	Has the patient been informed of the discharge plan and do they agree?	Yes / No / N/A
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Transition & Continuity

☐	Has a referral for outpatient therapy been placed if further skilled care is needed?	Yes / No / N/A
☐	Has the restorative nursing team been briefed and handed off the maintenance program (if applicable)?	Yes / No / N/A
☐	Has nursing been notified of any changes to assist level, weight-bearing status, or precautions at discharge?	Yes / No / N/A
☐	Has DME been ordered and confirmed available (walker, WC, grab bars, adaptive equipment)?	Yes / No / N/A
☐	Has the MDS team received updated GG discharge scores?	Yes / No / N/A

Documentation Complete

☐	Is the discharge note written with reason for discharge, initial vs final status, and goals addressed?	Yes / No / N/A
☐	Are all outstanding notes, charges, and co-signatures finalized?	Yes / No / N/A
☐	If maintenance — does the note fully document skilled need removal per Jimmo standard?	Yes / No / N/A
☐	Has the physician been notified of discharge if required by facility policy?	Yes / No / N/A

All YES or N/A?	■ YES — Safe to proceed with discharge	■ NO — Address outstanding items first
Therapist Signature:	_____	Date: _____
DOR Review:	_____	Date: _____

DOR responsibility: Review all planned discharges before they happen. Ensure the clinical rationale is sound, the documentation is complete, and the transition plan is in place. A premature discharge that results in re-hospitalization or patient harm is a compliance and liability issue. When in doubt — consult the IDT before finalizing the discharge decision.